

Study Staff Responsibilities and Workflow

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2.1 Overview

Study Managers and Assistant Study Managers at each hospital are the lead study staff members that ensure the success of this study. They are involved in all aspects of the study from identifying possible participants for recruitment through delivery of data and safeguarding participant files. Given the importance of the Study Managers and Assistant Study Manager positions, only those that attended the official training in January 2013 can fulfill the role of Study Manager and Assistant Study Manager for this study. If a Study Manager or Assistant Study Manager is male, they should refrain from conducting the Risk Factor Questionnaire interviews because of the sensitive nature of the some of the questions.

Interviewers at each hospital are also key to ensuring the success of this study. In most scenarios, they are the first person to talk to a potential participant about the details of the study. It is the Interviewers who need to garner trust from potential participants. They are responsible for recruiting women into the study, conducting the Risk Factor Questionnaire, taking anthropometry measurements, and overseeing some aspects of the biospecimen collections. Given the importance of the Interviewer position, only those Interviewers that attended the official Interviewer Training in January 2013 should fulfill the role of Interviewer for this study. If Interviewers leave the study and must be replaced, an experienced Interviewer at the sites should train the new Interviewers using the materials provided by Westat at the official training, and the new Interviewers should shadow experienced Interviewers until they are able to proceed on their own in the judgment of the Study Manager and the experienced Interviewer. All Interviewers should be female (no male Interviewers) because of the sensitive nature of the some of the questions in the Risk Factor Questionnaire.

Other hospital staff across different departments are also essential for this study. The route a woman takes to seek out a breast biopsy or breast surgery at one of the three hospital sites varies tremendously. Therefore, it is important to understand all of the possible ways a woman might enter the hospital's system in order to identify her as a potential participant for the study. All of these avenues must be actively monitored for accrual to ensure that all potential cases are approached. At KATH, women primarily present at the Breast Care Clinic. At PLH, women may self-present at the hospital, may present by referral, or may be ascertained through PLH's frequent

community outreach efforts. At KBTH, there are numerous potential points of presentation including the outpatient department breast clinic, surgery, radiotherapy, and oncology. It is essential at all sites that all women with suspicion of breast cancer or diagnosed with breast cancer within the past year be approached for participation in the study.

Once a woman is enrolled in the study, phlebotomists and laboratory technicians draw blood from participants and process it in a timely fashion. Surgeons conduct the breast tissue biopsies for cases enrolled in the study. Pathologists prepare and review the breast tissue samples and provide details about the diagnosis for this study.

Data entry staff use the data entry system provided by Westat to enter all responses and information collected on the Risk Factor Questionnaire and other study forms. Staff also use a scanner at each site to scan all study forms for receipt at Westat.

Staff responsibilities for Study Managers are described further in this chapter and additional chapters contained within the Manual of Operations and Procedures. Interviewer responsibilities are described in detail in the Interviewer's Manual. Other hospital staff responsibilities are described in detail in Chapters 7 - 12. Data Entry staff responsibilities are described in detail in Chapters 15 and 16.

2.2 Study Manager and Assistant Study Manager Responsibilities

Study Managers are responsible for a number of study tasks including but not limited to the following list. Study Managers are responsible for:

- Maintaining communications on a daily basis with hospital staff in different departments in order to identify potential cases for participation in the study.
- Managing the list of names of women provided by Dr. Biritwum to use for recruitment of controls for participation.
- Assigning Participants Identifiers (PIDs) to potential participants (both Cases and Controls).
- Assigning potential participants to Interviewers.
- Creating the Participant Folder.

- Overseeing the completion of study tasks, forms, and logs.
- Coordinating with Oncologists, Surgeons, Phlebotomists, and Pathologists to ensure study tasks are carried out and the forms are completed.
- Performing Quality Control of certain study tasks.
- Preparing questionnaire and forms for data entry, transfer, and storage.
- Maintaining and safeguarding participant files and all forms associated with each participant in the study.
- Updating the Recruitment Tracking Log and Supply Inventory and providing weekly to NCI and Westat.
- Coordinating with laboratory staff the packing biospecimens for shipment to the National Cancer Institute's biorepository in Frederick, MD, USA on an as needed basis.

2.2.1 Identifying Potential Cases and Managing their Recruitment

A potential Case is a woman who:

- Has been referred for a breast tissue biopsy by a doctor, nurse, or clinician (including a traditional healer);
- Self-presents without a referral for a breast tissue biopsy, but with symptoms considered suspicious for breast cancer;
- Has already had a breast tissue biopsy (either at KBTH, KATH, PLH, or another hospital), was diagnosed with invasive or *in situ* breast cancer in the past year, and is returning for non-surgical treatment; or
- Has already had a breast tissue biopsy (either at KBTH, KATH, PLH, or another hospital), was diagnosed with invasive or *in situ* breast cancer in the past year, and is returning for breast surgery.

It is expected that the majority of potential cases will be women who present to the hospital for a breast tissue biopsy. However, it is very important for this study that all potential Cases are sought out and approached to be in the study. Since potential Cases will be visiting different departments within a hospital (such as Oncology and Surgery), Study Managers will need to maintain communication on a daily basis with staff members in these departments in order to determine if there are potential Cases that need to be assigned to an Interviewer. The Study Manager should visit staff in these departments and explain the study and the importance of recruiting all potentially

eligible Cases. Copies of the study brochure should be given to hospital staff in different departments so that they can be given to potential cases when they arrive. If the Study Manager encounters resistance or difficulties in advertising the study in other hospital departments, they should immediately inform the PIs of the study at each hospital and at NCI. The PIs of the study will help identify ways to facilitate recruitment of all potential Cases across all departments in the hospital.

Once potential Cases have been identified, Study Managers assign Interviewers to determine their eligibility and recruit them into the study. Study Managers need to evaluate if the potential Case needs an Interviewer fluent in Twi. Twi is the only language other than English that Interviewers can use to administer the questionnaire to women. Refer to Chapter [3] in this manual for more information about determining a woman's eligibility in the study.

Study Managers should know the strengths of each Interviewer and should assign the recruitment of Cases to those Interviewers that can develop a good rapport and connection with the women, who are likely to be upset, sad, or scared about their breast disease. (Refer to Chapter 5 in the Interviewer Manual.)

2.2.2 Managing Recruitment of Potential Controls

For the feasibility phase of the study, potential Controls inside the defined catchment areas surrounding Accra and Kumasi were identified by Professor Biritwum and his staff at KBTH (refer to Chapter 3, Recruitment and Eligibility, for additional details about the selection of controls). A list of names of women who were potential Controls was given to the Study Managers. The Controls were matched 2:1 to Cases during the study's feasibility phase. However, Controls were identified and recruited at too fast a rate, such that the Controls were recruited much more quickly than Cases during the same time period.

Because this is a population based case-control study and the source of incident Cases is a dynamic population, the Controls needed for each Case must come from the same population and have a similar risk of developing breast disease during the same time-period as the time of the diagnosis of the Case. For the main part of the study, which began in September 2013, Controls are matched 1:1 to Cases and their recruitment into the study ideally should be at the same approximate rate as Cases. However, given the time required for enumeration for the main phase of the study and logistics involved with staff going to the field to approach Controls for participation, there may be

periods of time with no Control accrual and periods of time with more rapid accrual of Controls than Cases. It is important that all Controls on the lists provided to the Study Managers be fully pursued per the protocol in Chapter 3 of this manual, and the pace should not exceed the capacity of the Interviewers to manage the influx of Controls while still ensuring that sufficient resources are available to approach all potential Cases.

Potential Controls are approached at their residence, in person or by telephone, and are scheduled to come to the hospital for final eligibility determination and to complete the study. It is preferable to complete the study in the hospital if at all possible to enable collection of all biospecimens and anthropometry measurements. However, based on Control participation rates observed in the feasibility phase of the study, the protocol was revised to allow for Interviewers to conduct the study in the field for Controls if after 4 good faith attempts to encourage Controls to come to the hospital they are unable or unwilling to do so. Procedures for recruitment of Controls are described in greater detail in Chapter 3.

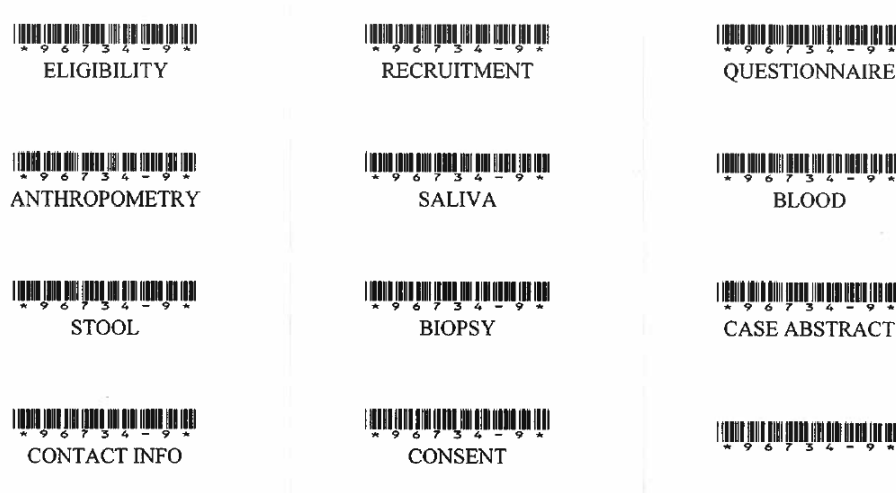
2.2.3 Assigning a Participant Identifier (PID) to a Potential Participant

All women identified as potential participants (both Cases and Controls) must be assigned a Participant Identifier (PID).

Only one PID is assigned to one woman. The PID has five digits followed by a dash and single digit (e.g. 12345-6). The following range of PIDs has been assigned to each hospital as shown:

KBTH	10001 - 19999
KATH	20001 - 29999
PLH	30001 – 39999

Sheets of PID labels have been provided to each hospital. One sheet of labels is for one PID (refer to Exhibit 2-1). Each label has a barcode, the PID, and in some cases the name of the form in which the PID label should be placed. There are many extra PID labels provided without any form names. These extra PID labels should be used as needed for organizing the participant file and additional paperwork.

Exhibit 2-1. Example of a sheet of labels for one PID

When a PID is assigned to a potential participant, the Study Manager should record the PID in the Recruitment Tracking Log (Appendix 2-1). The Recruitment Tracking Log should be updated on a daily basis as status reports about recruitment are requested on a weekly basis. For Controls, PIDs should be assigned as soon as the women are first selected from the control listing for approach.

2.2.4 Assigning Potential Participants to Interviewers

As previously stated, Study Managers should evaluate and be familiar with the individual strengths and talents of each Interviewer. A mixture of Cases and Controls should be interviewed by each Interviewer throughout the course of the study. Given that, certain Interviewers may be better matched to certain potential participants. For example, if an Interviewer has certain mannerisms where she is especially good at talking with and garnering trust from women who may be resistant to learning about the study because they are anxious or scared about an upcoming breast biopsy or surgery, then she would be a good choice for recruiting potential Cases. The Interviewer assigned to recruit the potential Cases should be the same Interviewer responsible for completing the Interviewer components of the study (i.e. conducting the Risk Factor Questionnaire).

Study Managers should assign the recruitment of Controls to those Interviewers that have the best likelihood of locating a potential Control and recruiting them into the study. If an Interviewer is fluent in reading and speaking Twi (and possibly other local languages) and is good at reading maps and locating households, then she would be a good choice for recruiting potential Controls.

Considerations should be made with regard to language, geography, and travel time to the hospital (i.e. where the woman lives) when assigning potential Controls to Interviewers.

The Interviewer's role in the recruitment of Controls will differ by site. It was initially intended that an Interviewer would first determine if a potential Control was eligible to participate in the study and then make arrangements for the Control to visit the hospital. In the feasibility phase of the study, however, in Accra the potential Controls were approached and recruited by Census Workers rather than by a trained study Interviewer. The Census Workers scheduled the participants' appointments at the hospital, and an Interviewer was assigned at the hospital visit to confirm eligibility and complete the study. In contrast, in the feasibility phase in Kumasi, Census workers were not used and the Interviewers approached and recruited potential Controls. For the main phase of the study, Interviewers should accompany the Census Workers in Accra whenever possible. This was demonstrated to be effective in increasing Control participation rates during refusal conversion efforts in the feasibility phase. When a Control comes to the hospital to complete the components of the study, a different Interviewer or the same Interviewer can be assigned to the woman at that time.

When deciding on the number of potential participants to assign to each Interviewer on a daily or weekly basis, the Study Manager should keep in mind the length of time needed to complete all components of the study. The approximate time needed from the time of recruitment through collecting the saliva sample and distributing the stool collection kit is approximately two hours.

Study Managers should prepare the Recruitment Outcome Form, Eligibility Screener Form, and the Contact Information Form for Interviewers by placing the correct PID label to each form. Any additional information needed for contacting and locating Controls should also be included. Each set of forms with PID labels should be neatly organized and distributed to Interviewers as they are assigned potential participants.

2.2.5 Creating the Participant Folder

For each potential participant assigned a PID, the Study Manager creates a participant folder. A yellow file folder is for Cases and a blue file folder is for Controls. A PID label should be placed on the file-folder tab to identify the participant's file folder. A self-adhesive paper fastener should be applied to the inside of the file folder, centered at the top on the left and right sides, as needed. In

some cases, very little paperwork may be collected for a potential participant. For example, if a woman is approached to be in the study but is not interested or cannot be found, then at the very minimum, she should have a completed Recruitment Outcome Form and the remaining unused PID labels in her participant folder. This is true for Cases and Controls. Other women may complete all components of the study and their participant file should be well organized. The Risk Factor Questionnaire should be secured to the participant folder using a binder clip until it is prepared for data entry (Chapter 16, Data Entry and Management).

2.2.6 Completion of Study Tasks, Forms, Logs, and Transmittals

Study Managers should be organized, take careful notes, and use the Recruitment Tracking Log to ensure that all study tasks are carried out and the appropriate forms have been completed. Many study tasks are carried out by the Interviewers. For Controls, Study Managers should review each Interviewer's Record of Calls (Appendix 2-2) to determine the status of recruitment of potential participants assigned to Interviewers. On a daily basis if required, the Study Manager records information from the Recruitment Outcome Form and the Record of Calls into the Recruitment Tracking Log. The Recruitment Tracking Log updates the PIs about recruitment on a weekly basis.

Other hospital staff carry out the blood draw, breast tissue biopsy, and complete the Breast Case Abstract Form. Study Managers coordinate with these departments to ensure the completion of study tasks and the return of completed forms. When Blood Processing was performed at KATH for PLH participants, the Study Manager coordinated transportation of the Blood Collection Form to and from KATH along with the blood sample, its placement in the correct participant folders, and submission for data entry.

Transmittals have been created to help track the status of certain tasks being performed by other hospital staff. These transmittals are referenced in following chapters in this MOOP with instructions for how they should be used.

Additionally, once all study tasks have been completed, the data entry staff will be responsible for entering data from the Risk Factor Questionnaire and study forms into the data entry system.

Study Managers will ultimately be responsible for overseeing and ensuring that all study tasks are carried out by appropriate study or hospital staff and all completed forms are returned to them.

2.2.7 Coordinating with Phlebotomists

When participants provide Informed Consent to participate in the study and agree to provide biospecimens, the Study Manager goes through the procedures outlined in Chapter 7, Preparing for Biospecimen Collections, in order to properly prepare the biospecimen collection forms and verify the contents of the biospecimen Master Kit. Once the contents of the Master Kit are verified and all biospecimen collection forms are prepared, the Interviewer collects the saliva and explains the stool collection procedures. The Study Manager needs to coordinate with hospital nurses and/or phlebotomists to ensure their availability to draw blood and send it to the laboratory for processing within the short permissible time-frame.

2.2.8 Coordinating with Surgical Staff (Cases Only)

Study Managers should coordinate with the surgical staff to ensure they are aware when a Case undergoing a breast biopsy or breast surgery is enrolled in the study. For Cases that enroll and agree to provide breast tissue, the correct biopsy needle and tissue processing supplies must be used (refer to Chapter 11 for details) and a Breast Tissue Biopsy Form is completed. Study Managers need to apply the correct PID label to the Breast Tissue Biopsy Form. If warranted, either a Study Manager or Interviewer accompanies the participant to surgery and delivers the Breast Tissue Biopsy Form to the surgical staff. Study Managers need to ensure they receive the completed Breast Tissue Biopsy Form for each enrolled Case who is biopsied or who undergoes a breast surgery.

2.2.9 Coordinating with Pathologists (Cases Only)

Pathologists review the breast tissue cores taken from Cases enrolled in the study (refer to Chapter 11 for details). Study Managers need to ensure the completion and return of the Breast Case Abstract Form by hospital pathology staff. Since PLH transfers their breast tissue biopsy cores to KBTH for processing, coordination by the Study Managers at each hospital is particularly important.

2.2.10 Performing Quality Control of Certain Study Tasks

Throughout the entire length of the study, Study Managers are responsible for checking approximately 5-10% of the study tasks for quality control. Study Managers should use each Interviewer's Record of Calls to evaluate if the appropriate number of attempts were made to recruit a potential Control into the study. A review of the Recruitment Outcome Form and Eligibility Screener Form should be done to check for missing or illogical responses. The Study Managers should clarify these with Interviewers at the earliest opportunity. Study Managers should shadow Interviewers on a regular basis when they are conducting interviews, taking Anthropometry measurements, collecting saliva, and explaining the stool collection procedures. Study Managers should also review the Informed Consent and check the Saliva, Stool, and Blood Forms to verify that the appropriate questions were asked. Study Managers should check the Breast Tissue Biopsy Form and Breast Case Abstract Form for completeness.

When an error or issue is identified, Study Managers should bring it to the attention of the staff member. Study Managers should review the correct procedures with the staff member and discuss any problems that may have resulted in the error. If the error is systemic across study staff and appears to be the result of misleading study instructions or a confusing form, the Study Manager should report this to Westat and NCI PIs immediately in order for revisions and/or clarifications to be made.

2.2.11 Preparing for Data Entry and Storage

As study tasks for a participant are completed, the Study Manager prepares the Risk Factor Questionnaire and other study forms for data entry. The Study Manager uses a transmittal to track the data entry process (refer to Chapter 16).

2.2.12 Safeguarding Participant Files and Participant Information

All participant files are to be kept in a filing cabinet with a lock when not in use. Participant files are to be grouped by the following status: (1) files with outstanding forms or information; (2) files ready for data entry; and (3) files returned from data entry. Participant files should not be left out unattended. Only those forms from within the file that need to be in other hospital departments

should be removed from the file. The entire participant file should never be given to any other hospital department and should remain in the study clinic.

When a participant file is ready for data entry, only the forms listed on the Data Entry Transmittal should be given to data entry staff. For example, the Contact Information form should not be sent for data entry because it contains personal identifying information that the data entry staff are not allowed to see. If for any reason, a participant name is written on a form for data entry, it should either be removed or made unreadable by crossing it out with a black marker. Participant names and contact information will not be entered into the data entry system, therefore the Informed Consent and Contact Information Sheet should always remain in the participant file (Refer to Chapter 15).

2.2.13 Reports to NCI and Westat

Study Managers email the Recruitment Tracking Log to NCI and Westat on a weekly basis. The Recruitment Tracking Log includes a Supply Inventory which is updated by the Study Manager weekly to accurately reflect the count of remaining supplies and forms of each type at each site. In addition, Study Managers should alert Westat as soon as any supply shortages are foreseen. As the study progresses, additional reports will be generated using data entered into the data entry system (Refer to Chapter 15, Reports, Monitoring and Quality Control).

Study Managers should maintain careful notes and use the checklists, logs, and transmittals provided in order to be able to answer questions from NCI and Westat.

2.2.14 Arranging for Biospecimen Packing and Shipping

NCI checks in with Study Managers and laboratory staff on a regular basis to determine the freezer space available, the number of specimens available to be shipped and the timing of biospecimen shipments. Separate shipments are arranged from Accra and from Kumasi with more frequent shipments from Kumasi during the time that KATH is processing and storing the blood samples collected at PLH. NCI and a contracted shipping company ensure proper shipment of biospecimens from Accra and Kumasi to the NCI Biorepository in Frederick, MD.